

Almond and Doyle Jr. Summary

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Broad Question

How does coverage for extended hospital stays affect postpartum health outcomes for mothers and infants? Further, does this extended coverage increase moral hazard, leading to longer hospital stays without corresponding health benefits?

Specific Question

Using an RDD, can we determine the causal impact of extended postpartum hospital stay coverage on maternal and infant health outcomes, and does this extended coverage lead to increased moral hazard in terms of longer hospital stays?

Why Should a Policymaker Care?

Policymakers should care about this question because postpartum care is crucial for the health and well-being of both mothers and infants. Extended hospital stays can provide necessary medical attention, support for breastfeeding, and education on newborn care. However, if extended coverage leads to moral hazard, it could result in unnecessary healthcare costs without improving health outcomes. Understanding the balance between adequate care and cost-effectiveness is essential for designing policies that optimize health benefits while managing healthcare expenditures.

Why Should an Economist Care?

Economists should care about this question because it touches on the principles of healthcare economics, particularly the concepts of moral hazard and cost-benefit analysis. The study provides an opportunity to analyze how insurance coverage influences behavior (in this case, hospital stay duration) and to assess the efficiency of healthcare resource allocation. Additionally, the use of a regression discontinuity design (RDD) offers a robust methodological approach to identify causal effects, which is of significant interest in econometric research.

Understanding these dynamics can inform broader discussions on healthcare policy, insurance design, and economic incentives in the healthcare system.

What do we learn from the paper?

The paper by Almond and Doyle Jr. (2011) investigates the impact of extended postpartum hospital stay coverage on maternal and infant health outcomes using a regression discontinuity design (RDD). The authors exploit a policy change that extended the minimum hospital stay for new mothers from 24 hours to 48 hours, allowing them to compare health outcomes before and after the policy implementation.

Empirical Strategy

The authors employ a regression discontinuity design (RDD) to identify the causal effects of extended postpartum hospital stay coverage. The key feature of their empirical strategy is the use of a cutoff point based on the timing of the policy change. Mothers who gave birth just before the policy change were subject to the 24-hour stay, while those who gave birth just after were eligible for the 48-hour stay. This creates a natural experiment where the only significant difference between the two groups is the length of hospital stay coverage.

Limitations and Solutions

Concerns of endogeneity from underlying health conditions extending hospital stays. This is overcome by the randomness of birth time and the reimbursement schedules of hospitals.